



NAME OF THE PRODUCT

Prega-75 (Pregabalin Capsule 75mg)
Prega-150 (Pregabalin capsules 150 mg)

NAME AND STRENGTH OF ACTIVE INGREDIENT

Prega-75 (Pregabalin Capsule 75mg)
Name: Pregabalin Strength: 75 mg
Prega-150 (Pregabalin capsules 150 mg)
Name: Pregabalin Strength: 150 mg

DOSAGE FORM

Oral

PRODUCT DESCRIPTION

Prega-75 (Pregabalin Capsule 75mg)
Orange cap/White body size ‘4’ hard gelatin capsules imprinted with ‘140’ on cap and ‘J’ on body with black ink, filled with white to off white powder.
Prega-150 (Pregabalin capsules 150 mg)
White cap/White body size ‘2’ hard gelatin capsules imprinted with ‘142’ on cap and ‘J’ on body with black ink, filled with white to off white powder.

PHARMACODYNAMICS

The active substance, pregabalin, is a gamma-aminobutyric acid analogue [(S)-3-(aminomethyl)- 5-methylhexanoic acid].
Mechanism of Action: Pregabalin binds to an auxiliary subunit (α2-δ protein) of voltage-gated calcium channels in the central nervous system.

PHARMACOKINETICS

Pregabalin steady-state pharmacokinetics is similar in healthy volunteers, patients with epilepsy receiving antiepileptic rugs and patients with chronic pain.

Absorption: Pregabalin is rapidly absorbed when administered in the fasted state, with peak plasma concentrations occurring within 1 hr following both single and multiple dose administration. Pregabalin oral bioavailability is estimated to be ≥90% and is independent of dose. Following repeated administration, steady state is achieved within 24-48 hrs. The rate of pregabalin absorption is decreased when given with food resulting in a decrease in C_{max} by approximately 25-30% and a delay in T_{max} to approximately 2.5 hrs. However, administration of pregabalin with food has no clinically significant effect on the extent of pregabalin absorption.

Distribution: Pregabalin has been shown to cross the blood brain barrier in mice, rats and monkeys. Pregabalin has been shown to cross the placenta in rats and is present in the milk of lactating rats. In humans, the apparent volume of distribution of pregabalin following oral administration is approximately 0.56 L/kg. Pregabalin is not bound to plasma proteins.

Metabolism: Pregabalin undergoes negligible metabolism in humans. Following a dose of radiolabelled pregabalin, approximately 98% of the radioactivity recovered in the urine was unchanged pregabalin. The N-methylated derivative of pregabalin, the major metabolite of pregabalin found in urine accounted for 0.9% of the dose. Preclinical, there was no indication of racemisation of pregabalin S-enantiomer to the R-enantiomer.

Elimination: Pregabalin is eliminated from the systemic circulation primarily by renal excretion as unchanged drug. Pregabalin mean elimination half-life is 6.3 hrs. Pregabalin plasma clearance and renal clearance are directly proportional to creatinine clearance. Dosage adjustment in patients with reduced renal function or undergoing haemodialysis is necessary.

Linearity/Nonlinearity: Pregabalin pharmacokinetics are linear over the recommended daily dose range. Intersubject pharmacokinetic variability for pregabalin is low (<20%). Multiple dose pharmacokinetics are predictable from single-dose data. Therefore, there is no need for routine monitoring of plasma concentrations of pregabalin.

Pharmacokinetics in Special Patient Groups:

Gender: Gender does not have a clinically significant influence on the plasma concentrations of pregabalin.

Renal Impairment: Pregabalin clearance is directly proportional to creatinine clearance. In addition, pregabalin is effectively removed from plasma by haemodialysis (following a 4-hr haemodialysis treatment, plasma pregabalin concentrations are reduced by approximately 50%). Because renal elimination is the major elimination pathway, dosage reduction in patients with renal impairment and dosage supplementation following haemodialysis is necessary.

Hepatic Impairment: No specific pharmacokinetic data was available in patients with impaired liver function. Since pregabalin does not undergo significant metabolism and is excreted predominantly as unchanged drug in the urine, impaired liver function would not be expected to significantly alter pregabalin plasma concentrations.

Elderly >65 years: Pregabalin clearance tends to decrease with increasing age. This decrease in pregabalin oral clearance is consistent with decreases in creatinine clearance associated with increasing age. Reduction of pregabalin dose may be required in patients who have age-related compromised renal function.

Breast-feeding mothers: Lactation had little to no influence on pregabalin pharmacokinetics. Pregabalin was excreted into breast milk with average steady-state concentrations approximately 76% of those in maternal plasma. The estimated average daily infant dose of pregabalin from breast milk (assuming mean milk consumption of 150 mL/kg/day) was 0.31 mg/kg/day, which on a mg/kg basis would be approximately 7% of the maternal dose.

INDICATIONS

Neuropathic Pain: Pregabalin is indicated for the treatment of peripheral and central neuropathic pain in adults.

Epilepsy: As adjunctive therapy in adults with partial seizures with or without secondary generalisation.

Generalised Anxiety Disorder: Treatment of Generalised Anxiety Disorder (GAD) in adults.

Fibromyalgia: Management of fibromyalgia.

RECOMMENDED DOSE

The dose range is 150-600 mg/day given in either 2 or 3 divided doses. Pregabalin may be taken with or without food.

Neuropathic Pain: Pregabalin treatment can be started at a dose of 150 mg/day. Based on individual patient response and tolerability, the dosage may be increased to 300 mg/day after an interval of 3-7 days, and if needed, to a maximum dose of 600 mg/day after an additional 7-day interval.

Epilepsy: Pregabalin treatment can be started with a dose of 150 mg/day. Based on individual patient response and tolerability, the dosage may be increased to 300 mg/day after 1 week. The maximum dosage of 600 mg/day may be achieved after an additional week.

Generalised Anxiety Disorder: The dose range is 150-600 mg/day given as 2 or 3 divided doses. The need for treatment should be reassessed regularly. Pregabalin treatment can be started with a dose of 150 mg/day. Based on individual patient response and tolerability, the dosage may be increased to 300 mg/day after 1 week. Following an additional week, the dosage may be increased to 450 mg/day. The maximum dosage of 600 mg/day may be achieved after an additional week.

Fibromyalgia: The usual dose range for most patients is 300 to 450 mg per day given in two divided doses. Some patients may derive additional benefit at 600 mg per day. Dosing should begin at 75 mg two times a day (150 mg/day) and may be increased to 150 mg two times a day (300 mg/day) within 1 week based on efficacy and tolerability. Patients who do not experience sufficient benefit with 300 mg/day may be further increased to 225 mg two times a day (450 mg/day). If needed, in some patients, based on individual response and tolerability, the dose may be increased to maximum dosage of 600 mg/day after an additional week.

Patients with Renal Impairment: Pregabalin is eliminated from the systemic circulation primarily by renal excretion as unchanged drug. As pregabalin clearance is directly proportional to creatinine clearance, dosage reduction in patients with compromised renal function must be individualised according to creatinine clearance (CrCl), as indicated in the table determined using the following formula.

$$CL_{cr} \text{ (mL/min)} = \left[\frac{[140 - \text{age (years)}] \times \text{weight (kg)}}{72 \times \text{serum creatinine (mg/dL)}} \right] (\times 0.85 \text{ for female patients})$$

Pregabalin is removed effectively from plasma by haemodialysis (50% of drug in 4 hrs). For patients receiving haemodialysis, the pregabalin daily dose should be adjusted based on renal function. In addition to the daily dose, a supplementary dose should be given immediately following every 4-hr haemodialysis treatment (see table).

Pregabalin Dosage Adjustment Based on Renal Function			
Creatinine clearance (CL _{cr}) (mL/min)	Total Pregabalin Daily Dose*		Dose Regimen
	Starting Dose (mg/day)	Maximum Dose (mg/day)	
≥ 60	150	600	BID or TID
≥ 30 to < 60	75	300	BID or TID
≥ 15 to < 30	25-50	150	Once daily or BID
< 15	25	75	Once daily
Supplementary dosage following haemodialysis (mg)			
	25	100	#Single dose
TID: 3 divided doses; BID: 2 divided doses *Total daily dose (mg/day) should be divided as indicated by dosage regimen to provide mg/ dose # Supplementary dose is a single additional dose			

Patients with Hepatic Impairment: No dosage adjustment is required for patients with hepatic impairment.

Children and Adolescents: Pregabalin is not recommended for use in children <12 years and adolescents (12-17 years) due to insufficient data on safety and efficacy.

Elderly (>65 years): Elderly patients may require a dose reduction of pregabalin due to decreased renal function
Discontinuation of Pregabalin: In accordance with current clinical practice, if pregabalin has to be discontinued, it is recommended that this should be done gradually over a minimum of 1 week independent of the indication.

MODE OF ADMINISTRATION

Oral route of administration.

CONTRAINDICATIONS

Hypersensitivity to pregabalin or to any of the excipients of pregabalin.

WARNINGS AND PRECAUTIONS

Angioedema: There have been reports of angioedema in patients during initial and chronic treatment with pregabalin. Specific symptoms included swelling of the face, mouth (tongue, lips and gums), and neck (throat and larynx). There were reports of life-threatening angioedema with respiratory compromise requiring emergency treatment. Pregabalin should be discontinued immediately in patients with these symptoms.

Caution should be exercised when prescribing pregabalin to patients who have had a previous episode of angioedema. In addition, patients who are taking other drugs associated with angioedema (eg, angiotensin converting enzyme ACE inhibitors may be at increased risk of developing angioedema.

Hypersensitivity: There have been reports of hypersensitivity in patients shortly after initiation of treatment with pregabalin. Adverse reactions included skin redness, blisters, hives, rash, dyspnea and wheezing. Pregabalin should be discontinued immediately in patients with these symptoms.

Suicidal Behavior and Ideation:

Antiepileptic drugs (AEDs), including pregabalin, increase the risk of suicidal thoughts or behavior in patients taking these drugs for any indication. Monitor patients treated with any AED for any indication for the emergence or worsening of depression, suicidal thoughts or behavior, and/or any unusual changes in mood or behavior. Anyone considering prescribing pregabalin or any other AED must balance the risk of suicidal thoughts or behavior with the risk of untreated illness. Epilepsy and many other illnesses for which AEDs are prescribed are themselves associated with morbidity and mortality and an increased risk of suicidal thoughts and behavior. Should suicidal thoughts and behavior emerge during treatment, the prescriber needs to consider whether the emergence of these symptoms in any given patient may be related to the illness being treated.

Respiratory Depression: There is evidence from case reports, human studies, and animal studies associating pregabalin with serious, life-threatening, or fatal respiratory depression when co-administered with central nervous system (CNS) depressants, including opioids, or in the setting of underlying respiratory impairment. When the decision is made to co-prescribe pregabalin with another CNS depressant, particularly an opioid, or to prescribe pregabalin to patients with underlying respiratory impairment, monitor patients for symptoms of respiratory depression and sedation, and consider initiating pregabalin at a low dose. The management of respiratory depression may include close observation, supportive measures, and reduction or withdrawal of CNS depressants (including pregabalin). There is more limited evidence from case reports, animal studies, and human studies associating pregabalin with serious respiratory depression, without co-administered CNS depressants or without underlying respiratory impairment.

Dizziness and Somnolence: Pregabalin may cause dizziness and somnolence. Inform patients that pregabalin-related dizziness and somnolence may impair their ability to perform tasks such as driving or operating machinery and could increase the occurrence of accidental injury (fall) in the elderly population. There have also been post-marketing reports of loss of consciousness, confusion and mental impairment. Therefore, patients should be advised to exercise caution until they are familiar with the potential effects of the medication. Pregabalin is not known to be active at receptor sites associated with drugs of abuse. Cases of misuse, abuse and dependence have been reported in the post-marketing database. As with any CNS active drug, carefully evaluate patients for history of drug abuse and/or psychiatric disorders. Patients should be observed for signs of pregabalin misuse, abuse or dependence (e.g. development of tolerance, dose escalation, drug-seeking behavior).

Increased Risk of Adverse Reactions with Abrupt or Rapid Discontinuation: As with all antiepileptic drugs (AEDs), withdraw pregabalin gradually to minimize the potential of increased seizure frequency in patients with seizure disorders. Following abrupt or rapid discontinuation of Pregabalin, some patients reported symptoms including insomnia, nausea, headache, anxiety, hyperhidrosis, and diarrhea. If Pregabalin is discontinued, taper the drug gradually over a minimum of 1 week rather than discontinue the drug abruptly.

Peripheral Edema: Pregabalin treatment may cause peripheral edema. In short-term administration of patients without clinically significant heart or peripheral vascular disease, there was no apparent association between peripheral edema and cardiovascular complications e.g., hypertension or congestive heart failure. Peripheral edema was not associated with laboratory changes suggestive of deterioration in renal or hepatic function. Higher frequencies of weight gain and peripheral edema were observed in patients taking both pregabalin and a thiazolidinedione antidiabetic agent compared to patients taking either drug alone. Peripheral edema was reported in patients who were using thiazolidinedione antidiabetic agents only, pregabalin only, and on both pregabalin and thiazolidinedione antidiabetic agents. Similarly, weight gain was reported in patients on pregabalin only; and in patients on both drugs. As the thiazolidinedione class of antidiabetic drugs can cause weight gain and/or fluid retention, possibly exacerbating or leading to heart failure, care should be taken when co-administering pregabalin and these agents. Because there are limited data on congestive heart failure patients with New York Heart Association (NYHA) Class III or IV cardiac status, pregabalin should be used with caution in these patients.

Weight Gain: Pregabalin treatment may cause weight gain. Pregabalin associated weight gain was related to dose and duration of exposure, but did not appear to be associated with baseline BMI, gender or age. Weight gain was not limited to patients with edema. Although weight gain was not associated with clinically important changes in blood pressure in short-term, the long-term cardiovascular effects of pregabalin-associated weight gain are unknown. While the effects of pregabalin-associated weight gain on glycemic control have not been systematically assessed, in controlled and longer-term administration with diabetic patients, pregabalin treatment did not appear to be associated with loss of glycemic control (as measured by HBA1c). In accordance with current clinical practice, some diabetic patients who gain weight on pregabalin treatment may need to adjust hypoglycaemic medications.

Abrupt or Rapid Discontinuation: Following abrupt or rapid discontinuation of short-term and long-term treatment with pregabalin, withdrawal symptoms have been observed in some patients. The following events have been mentioned: insomnia, headache, nausea, anxiety, diarrhoea, flu syndrome, nervousness, depression, pain, convulsion, hyperhidrosis and dizziness. The patient should be informed about this at the start of the treatment. Convulsions, including status epilepticus and grand mal convulsions, may occur during pregabalin use or shortly after discontinuing pregabalin. Concerning discontinuation of long-term treatment of pregabalin, data suggested that the incidence and severity of withdrawal symptoms may be dose-related. There are insufficient data for the withdrawal of concomitant antiepileptic medicinal products, once seizure control with pregabalin in the add-on situation has been reached, in order to reach monotherapy on pregabalin.

Dimensions	280 x 400 mm (Book Fold: 40 x 60 mm)
Customer/Country	Camber / Malaysia
Spec	Printed on 40 gsm bible paper
Pantone Colours	Black
Pharma Codes	Front Side: 11399 and Back Side: 11400

Note: Pharma Code Position will be change based on Machine folding feasibility.

Version: 00





Tumorigenic Potential: In standard preclinical *in vivo* lifetime carcinogenicity data of pregabalin, an unexpectedly high incidence of hemangiosarcoma was identified in 2 different strains of mice. The clinical significance of this finding is unknown. Clinical experience during pregabalin’s premarketing development provides no direct means to assess its potential for inducing tumors in humans.

Ophthalmological Effects: Although the clinical significance of the ophthalmologic findings is unknown, inform patients to notify their physician if changes in vision occur. If visual disturbance persists, consider further assessment. Consider more frequent assessment for patients who are already routinely monitored for ocular conditions. Visual adverse reactions have been reported, including loss of vision, visual blurring or other changes of visual acuity, many of which were transient. Discontinuation of pregabalin may result in resolution or improvement of these visual symptoms.

Creatine Kinase Elevations: Pregabalin treatment was associated with creatine kinase elevations. The relationship between these myopathy events and pregabalin is not completely understood because the cases had documented factors that may have caused or contributed to these events. Prescribers should instruct patients to promptly report unexplained muscle pain, tenderness, or weakness, particularly if these muscle symptoms are accompanied by malaise or fever. Pregabalin treatment should be discontinued if myopathy is diagnosed or suspected or if markedly elevated creatine kinase levels occur.

Decreased Platelet Count: Pregabalin treatment was associated with a decrease in platelet count. Pregabalin was not associated with an increase in bleeding-related adverse reactions.

PR Interval Prolongation: Pregabalin treatment was associated with PR interval prolongation.

Others: Although the effects of discontinuation on the reversibility of renal failure have not been systematically assessed, improved renal failure following discontinuation or dose reduction of pregabalin has been reported.

Misuse, abuse potential or dependence: Cases of misuse, abuse and dependence have been reported. Caution should be exercised in patients with a history of substance abuse and the patient should be monitored for symptoms of pregabalin misuse, abuse or dependence (development of tolerance, dose escalation, drug seeking behaviour have been reported).

Encephalopathy: Cases of encephalopathy have been reported, mostly in patients with underlying conditions that may precipitate encephalopathy.

Lactose intolerance: Pregabalin contains lactose monohydrate. Patients with rare hereditary problems of galactose intolerance, the Lapp-lactase deficiency or glucose-galactose malabsorption should not take this medicinal product.

Treatment of central neuropathic pain due to spinal cord injury: In the treatment of central neuropathic pain due to spinal cord injury the incidence of adverse reactions in general, central nervous system adverse reactions and especially somnolence was increased. This may be attributed to an additive effect due to concomitant medicinal products (e.g., anti-spasticity agents) needed for this condition. This should be considered when prescribing pregabalin in this condition.

Effects on the Ability to Drive or Operate Machinery: Pregabalin may have minor or moderate influence on the ability to drive and use machines. Pregabalin may cause dizziness and somnolence and therefore may influence the ability to drive or use machines. Patients are advised not to drive, operate complex machinery or engage in other potentially hazardous activities until it is known whether this medication affects their ability to perform these activities.

INTERACTION WITH OTHER MEDICAMENTS

Since pregabalin is predominantly excreted unchanged in the urine, undergoes negligible metabolism in humans (<2% of a dose recovered in urine as metabolites), does not inhibit drug metabolism in vitro, and is not bound to plasma proteins, it is unlikely to produce, or be subject to, pharmacokinetic interactions.

Accordingly, in in vivo studies no clinically relevant pharmacokinetic interactions were observed between pregabalin and phenytoin, carbamazepine, valproic acid, lamotrigine, gabapentin, lorazepam, oxycodone or ethanol. Population pharmacokinetic analysis indicated that oral antidiabetics, diuretics, insulin, phenobarbital, tiagabine and topiramate had no clinically significant effect on pregabalin clearance.

Co-administration of pregabalin with the oral contraceptives norethisterone and/or ethinyl estradiol does not influence the steady-state pharmacokinetics of either substance. Pregabalin may potentiate the effects of ethanol and lorazepam. In controlled clinical trials, multiple oral doses of pregabalin coadministered with oxycodone, lorazepam, or ethanol did not result in clinically important effects on respiration. Pregabalin appears to be additive in the impairment of cognitive and gross motor function caused by oxycodone.

In the post-marketing experience, there are reports of respiratory failure, coma and deaths in patients taking pregabalin and other CNS depressant medications including in patients who are substance abusers. There are post-marketing reports of events related to reduced lower gastrointestinal tract function (e.g., intestinal obstruction, paralytic ileus, constipation) when pregabalin was coadministered with medications that have the potential to produce constipation, such as opioid analgesics.

No specific pharmacodynamic interaction studies were conducted in elderly volunteers.

Incompatibilities: Not applicable

PREGNANCY AND LACTATION

There is a limited amount of data on the use of pregabalin in pregnant women. Birth and postnatal neurodevelopmental outcomes. There were no statistically significant findings for stillbirth, low birth weight, preterm birth, small for gestational age, low Apgar score, and microcephaly. In pediatric population exposed in utero, the study did not provide evidence of an increased risk for attention deficit hyperactivity disorder (ADHD), autism spectrum disorders (ASD), and intellectual disabilities. Pregabalin use in the first trimester of pregnancy may cause major birth defects in the unborn child. Pregabalin should not be used during pregnancy unless the benefit to the mother clearly outweighs the potential risk to the fetus. Women of childbearing potential must use effective contraception during treatment.

Pregabalin is excreted in the milk of lactating women. As the safety of pregabalin in infants is not known, breast-feeding is not recommended during treatment with pregabalin. A decision must be made whether to discontinue breast-feeding or to discontinue from pregabalin therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

SIDE EFFECTS

The most commonly reported adverse reactions were dizziness and somnolence. Adverse reactions were usually mild to moderate in intensity. The most common adverse reactions resulting in discontinuation from pregabalin treatment groups were dizziness and somnolence. The adverse reactions listed may also be associated with the underlying disease and/or concomitant medications.

System Class	Organ	Adverse Drug Reaction
Infections and infestations		
Common		Nasopharyngitis
Blood and lymphatic system disorders		
Uncommon		Neutropenia
Metabolism and nutrition disorders		
Common		Appetite increased
Uncommon		Anorexia, hypoglycemia
Psychiatric disorders		
Common		Euphoric mood, confusion, irritability, depression, disorientation, insomnia, libido decreased
Uncommon		Hallucination, restlessness, agitation, depressed mood, elevated mood, mood swings, depersonalization, abnormal dreams, word finding difficulty, libido increased, anorgasmia
Rare		Panic attack, disinhibition, apathy
Nervous system disorders		
Very common		Dizziness, somnolence
Common		Ataxia, coordination abnormal, tremor, dysarthria, amnesia, memory impairment, disturbance in attention, paraesthesia, hypoesthesia, sedation, balance disorder, lethargy

Uncommon	Syncope, myoclonus, psychomotor hyperactivity, dyskinesia, dizziness postural, intention tremor, nystagmus, cognitive disorder, speech disorder, hyporeflexia, hyperaesthesia, burning sensation
Rare	Stupor, parosmia, hypokinesia, ageusia, dysgraphia
Eye disorders	
Common	Vision blurred, diplopia
Uncommon	Peripheral vision loss, visual disturbance, eye swelling, visual field defect, visual acuity reduced, eye pain, asthenopia, photopsia, dry eye, lacrimation increased, eye irritation
Rare	Oscillopsia, altered visual depth perception, mydriasis, strabismus, visual brightness
Ear and labyrinth disorders	
Common	Vertigo
Uncommon	Hyperacusis
Cardiac disorders	
Uncommon	Tachycardia, atrioventricular block first degree, sinus bradycardia
Rare	Sinus tachycardia, sinus arrhythmia
Vascular disorders	
Uncommon	Hypotension, hypertension, hot flushes, flushing, peripheral coldness
Respiratory, thoracic and mediational disorders	
Uncommon	Dyspnoea, epistaxis, cough, nasal congestion, rhinitis, snoring
Rare	Throat tightness, nasal dryness
Gastrointestinal disorders	
Common	Vomiting, constipation, flatulence, abdominal distension, dry mouth
Uncommon	Gastrooesophageal reflux disease, salivary hypersecretion, hypoesthesia oral
Rare	Ascites, pancreatitis, dysphagia
Skin and subcutaneous tissue disorders	
Uncommon	Rash papular, urticaria, sweating
Rare	Cold sweat, Stevens-Johnson syndrome
Musculoskeletal and connective tissue disorders	
Common	Muscle cramp, arthralgia, back pain, pain in limb, cervical spasm
Uncommon	Joint swelling, myalgia, muscle twitching, neck pain, muscle stiffness
Rare	Rhabdomyolysis
Renal and urinary disorders	
Uncommon	Urinary incontinence, dysuria
Rare	Renal failure, oliguria
Reproductive system and breast disorders	
Uncommon	Erectile dysfunction, sexual dysfunction, ejaculation delayed, dysmenorrhoea
Rare	Breast pain, amenorrhoea, breast discharge, breast enlargement
General disorders and administration site conditions	
Common	Oedema peripheral, oedema, gait abnormal, fall, feeling drunk, feeling abnormal, fatigue
Uncommon	Generalised oedema, chest tightness, pain, pyrexia, thirst, chills, asthenia
Investigations	
Common	Weight increased
Uncommon	Blood creatine phosphokinase increased, alanine aminotransferase increased, aspartate aminotransferase increased, blood glucose increased, platelet count decreased, blood potassium decreased, weight decreased
Rare	White blood cell count decreased, blood creatinine increased

The following adverse drug reactions were reported during POST-MARKETING SURVEILLANCE:
Immune system disorder: *Uncommon:* Hypersensitivity; *Rare:* Angioedema, allergic reaction.
Nervous system disorders: *Very Common:* Headache; *Uncommon:* Loss of consciousness, mental impairment.
Eye disorders: *Rare:* Keratitis.
Cardiac disorders: *Rare:* Congestive heart failure.
Respiratory, thoracic and mediastinal disorders: *Rare:* Pulmonary oedema.
Gastrointestinal disorders: *Common:* Nausea, diarrhoea; *Rare:* Swollen tongue.
Skin and subcutaneous tissue disorders: *Uncommon:* Face swelling, pruritus; *Rare:* StevensJohnson syndrome.
Renal and urinary disorders: *Rare:* Urinary retention.
Reproductive system and breast disorders: *Rare:* Gynaecomastia.
General disorders and administration site conditions: *Uncommon:* Malaise.

SYMPTOMS AND TREATMENT OF OVERDOSE
In overdoses up to 15 g, no unexpected adverse reactions were reported. The most commonly reported adverse events observed when pregabalin was taken in overdose included affective disorder, somnolence, confusional state, depression, agitation and restlessness. Treatment of pregabalin overdose should include general supportive measures and may include haemodialysis if necessary.

STORAGE CONDITION
Store below 30°C and protect from moisture.

SHELF LIFE
3 years

DOSAGE FORMS AND PACKAGING AVAILABLE
10 x 10’s capsule in blister pack

PRODUCT REGISTRATION HOLDER
Camber Laboratories Sdn Bhd
E-13A-02, Menara SUEZCAP 2, KL Gateway,
No. 2, Jalan Kerinchi, Gerbang Kerinchi Lestari,
59200 Kuala Lumpur.

MANUFACTURER
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